Handcuffs Aren't Housing

Advocacy Toolkit: Coercion in California's Behavioral Health Policy

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Part 1: Issue Summary

California is adding legal pathways to compel mental health treatment. This toolkit shows where those pathways stand, what the evidence says about court-ordered and voluntary care, and what to ask your local leaders to do.

The ask: Protect funding for voluntary, community-based and peer-run services before expanding court-ordered care further.

The coercion pipeline

Step 1: Criminal penalties (Proposition 36). Proposition 36, approved by voters in November 2024, raises penalties for some theft and drug crimes. It creates a treatment-mandated felony for certain repeat drug possession: people who finish treatment have the charge dismissed, and those who do not can face up to three years in state prison.

Step 2: Narrower diversion (AB 46). AB 46 (Nguyen), signed June 29, 2026, amends Penal Code section 1001.36, the pretrial mental health diversion law. The Governor's office describes it as restoring broader judicial discretion to deny diversion. It also requires a court that denies diversion to state its reasons on the record.

Step 3: Court pathways, and where the bills landed. Of the four bills below, SB 989 became law and three did not advance.

Table 1. Court-pathway bills, 2025–26 session

Measure	What it does	Outcome
SB 989	Lets a first responder ask the county behavioral health agency to file a CARE petition; the agency must tell the responder what it decides.	Signed. Chapter 667, Statutes of 2026 (September 27, 2026).
SB 28	Would have strengthened the link between CARE Court and conservatorship.	Died in Assembly Appropriations (held August 13, 2026).
SB 1221	Would have barred courts from finding that a person can meet basic needs because jail provides them, let district attorneys attend gravely disabled hearings, and let counties with 750,000 or more residents prioritize state hospital placement for some conservatees.	Held in committee (May 14, 2026).
AB 1676	Would have let a county behavioral health director petition for an involuntary medication order, alone or with an assisted outpatient treatment (AOT) petition.	Did not advance. First hearing canceled at the author's request (April 13, 2026).

Step 4: The risk. A recurring cycle of institutional dependency, in which the main response to homelessness risks becoming locked facility placement. Proposals like the three that stalled could return as new bills in the next legislative session.

The Racial Justice Concern: Diagnosis as a Legal Trigger

Court-ordered treatment often depends on a diagnosis, and diagnoses are not applied evenly. Expanding who can be compelled into treatment risks building existing diagnostic gaps into law.

- **Diagnoses differ by race.** A review of 24 years of research found that Black consumers of mental health services received psychotic disorder diagnoses at roughly three to four times the rate of white consumers. The authors caution that this does not show higher true rates and that clinician bias may contribute. At one outpatient clinic, Black patients diagnosed with schizophrenia were more likely than white patients to screen positive for major depression, and the authors suggest clinicians may underweight mood symptoms in Black patients.
- **A diagnosis can decide eligibility.** California's CARE Act requires a diagnosis in the schizophrenia spectrum and other psychotic disorders class, or bipolar I disorder with psychotic features. A gap in diagnosis can therefore become a gap in who is eligible for court-ordered care.
- **Resources.** Prioritizing court-mandated and institutional placements risks shifting public funds away from permanent supportive housing and community care.
- **Consistency.** The state has committed to civil rights and health equity. Expanding involuntary frameworks while making that commitment risks mirroring punitive approaches to poverty management.

Follow the Data, Not the Fear

The evidence on both court-ordered and voluntary care is real but limited. Policymakers should weigh it with its limits stated.

- **Peer respite (New York City).** A Medicaid claims study compared 401 clients of a peer-staffed crisis respite program with a matched group of 1,796 people who had psychiatric hospital stays. Clients had fewer hospitalizations and about $2,138 less Medicaid spending per enrolled month in the first year. The differences were not significant in year two, program costs were not counted, and broader evidence on peer respite remains limited.
- **Court-ordered outpatient treatment (New York).** The independent evaluation of Kendra's Law, by researchers at the University of Pittsburgh and the Human Services Research Institute (HSRI), compared court-ordered AOT with voluntary Assertive Community Treatment (ACT). Both groups improved on several measures. Press coverage of the report says AOT did better on four of ten outcomes, ACT on two, and neither on four. The researchers noted that participants' improvements overlapped with enhanced access to intensive wraparound services. State officials read the findings as showing an advantage for AOT among a small eligible group; the evaluation's authors dispute that reading.

- **Laura's Law (California).** The Department of Health Care Services (DHCS) reported on AOT for July 2023 to June 2024. Among 172 court-involved participants in 16 counties, law enforcement contacts fell 30 percent, hospitalization fell 47 percent and homelessness fell 45 percent during enrollment, compared with before. The report says these improvements cannot be exclusively linked to AOT program services, and it notes limits including small numbers, self-reported data and differing county data systems.

The data do not settle whether court orders, or the services that come with them, drive these improvements. That uncertainty is a reason to fund and evaluate voluntary alternatives before expanding court-ordered care.

Part 2: Talking Points

Four statements for public comment at a Board of Supervisors meeting or a meeting with legislators.

1. **The racial justice concern.** Research finds that Black consumers of mental health services receive psychotic disorder diagnoses at roughly three to four times the rate of white consumers, and the researchers say clinician bias may be part of the reason. California's CARE Act eligibility depends on those diagnoses. If we expand who can be compelled into treatment, we risk turning a diagnostic gap into a legal one. That is a racial justice problem, and I am asking this Board to treat it as one.
2. **Policy consistency.** State policy should match its stated commitment to equity and civil liberties. Growing reliance on court-mandated interventions contradicts that commitment. Priorities should center on supportive housing and community care.
3. **The closing pipeline.** This year the Legislature passed AB 46, which broadens judges' discretion to deny mental health diversion, and SB 989, which lets first responders ask counties to start CARE petitions. Three bills that would have expanded court-ordered care, SB 28, SB 1221 and AB 1676, stalled. Similar proposals could return as new bills. We should keep voluntary community support at the center before they do.
4. **The ask.** Stop using anecdotes to justify state force. The evidence on court-ordered care is limited, and the state's own Laura's Law report says its improvements cannot be exclusively linked to AOT services. I am asking this Board to protect funding for voluntary, community-led services and to ask for outcome data before supporting any expansion of court-ordered care.

Part 3: Take Action

The Legislature has finished acting on this session's bills. Advocates can use this period to engage local Boards of Supervisors and state legislators, review these developments, and champion voluntary, community-based care ahead of the next session.

Find your meeting in 30 seconds

1. Search for your county's name and "Board of Supervisors" to reach its official website.
2. Look for one of three words: "Agendas" (what the Board will vote on), "Calendar" (when to show up) or "Board of Supervisors" (the main department page).

3. Read the agenda for behavioral health, mental health services or budget items, and the posted public comment procedures. Meeting days and times vary by county.

Questions to ask your Board

- What share of county behavioral health spending goes to voluntary and peer-run services, and how is that changing?
- What outcome data does the county collect on those services?
- How will the county fund and evaluate voluntary alternatives before supporting expanded court-ordered care?

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